Lung Transplant

Analytics Report · October 2026 Update

National Insights · ALUA Program Analysis · Policy Impact · Forecasts

PART 1 — NATIONAL ANALYSIS

- National volume 2005–2026 (Jan–Sep)
- DSA & CAS policy impact
- Program quadrant analysis
- Equity & access metrics

PART 2 — ALUA PROGRAM

- Volume history & Southeast peers
- SRTR PSR Jul 2026 & UNOS benchmarks
- Rolling HR surveillance (Oct 4, 2026)
- Offer acceptance, regression & Monte Carlo

OPTN 2005–Sep 30, 2026 (as of Oct 25, 2026) · SRTR PSR July 7, 2026 · Rolling reports Oct 4, 2026 · Offer acceptance Jan–Aug 2026

Web-scraped data

Personal project to apply text /data mining tools with AI help

OPTN-HRSA website:

Used AI-assisted coding to web-scrape lung transplant data for 54 US centers (active adult programs > 3 years)

Period 2005 – Sep 30, 2026 (OPTN data as of Oct 25, 2026)

Key Questions:

How has the U.S. lung transplant system changed over the past 20 years?

How did major allocation policy reforms — DSA removal and CAS implementation — redistribute volume and access across centers?

Where does ALUA stand now — nationally and against Southeast peers — and what are the major surveillance risks for 2026–2028?

Lung transplants by center — OPTN/HRSA, scraped Oct 26, 2026 (2026 = Jan 1–Sep 30)

Code	Center	2026*	2025	2024	2023	2022	2021	2020	2019	2018	2017	2016	2015	2014	2013	2012	2011	2010	2009	2008	2007	2006	2005
ALUA	University of Alabama Hospital	23	31	18	23	25	27	13	8	19	18	15	17	21	24	18	23	18	12	26	27	34	20
AZSJ	St. Joseph's Hospital and Medical Center	72	80	103	90	78	97	94	119	104	89	114	93	73	70	56	42	32	15	10	11		
AZUA	Banner University Medical Center-Tucson		2	16	15	20	17	36	31	41	28	9	8		1	2	26	15	14	21	17	17	6
CACS	Cedars-Sinai Medical Center	68	93	85	80	74	60	18	17	21	15	16	15	22	20	14	15	19	20	17	26	11	16
CASD	University of California San Diego Medica...	37	54	50	59	37	46	30	36	23	22	14	20	16	23	16	21	19	15	11	11	14	25
CASF	University of California San Francisco Me...	74	89	101	95	77	74	80	66	54	44	68	45	48	41	36	41	47	33	41	35	29	32
CASU	Stanford Health Care	55	70	67	72	55	54	59	35	49	58	38	42	31	32	45	45	26	38	42	39	28	12
CAUC	University of California at Los Angeles M...	69	83	92	82	72	70	120	91	78	73	103	90	71	79	72	57	50	52	54	59	60	54
CAUH	Keck Hospital of USC	20	21	23	27	31	14	33	25	16	13	12	16	16	15	7	8	11	23	20	17	19	37
COUC	University of Colorado Hospital/Health Sc...	32	46	45	51	40	27	32	44	44	30	27	23	28	30	25	28	27	25	28	37	28	42
FLFH	AdventHealth Orlando	26	31	9	17	13	25	22	16	16	20	18	3	1	14	9							
FLJM	Jackson Memorial Hospital University of M...	60	85	60	79	75	21	19	22	24	10	19	13	1	3	2	12	8	8	4	9	8	9
FLSL	Mayo Clinic Hospital Florida	42	69	79	54	37	39	39	48	41	40	43	41	34	42	45	44	44	42	40	35	26	32
FLTG	Tampa General Hospital	60	42	42	39	52	48	34	34	43	55	40	43	38	33	36	56	58	38	37	41	36	17
FLUF	UF Health Shands Hospital	69	65	73	52	49	68	93	90	70	56	45	37	12	28	28	36	29	32	36	34	53	37
GAEM	Emory University Hospital	52	44	29	24	20	14	17	25	9	20	19	20	25	32	34	29	31	35	27	25	27	26
ILLU	Loyola University Medical Center	30	34	40	46	48	32	34	47	47	39	40	45	51	30	21	36	39	42	46	39	26	38
ILUC	University of Chicago Medical Center	25	24	9	4	9	24	7	13	26	26	17	21	25	22	12	8	13	15	14	15	8	
IAIV	University of Iowa Hospitals and Clinics ...	15	19	17	18	24	26	23	15	17	11	15	16	12	16	14	14	20	15	12	5		
KYUK	University of Kentucky Medical Center	9	14	9	35	31	25	14	29	20	23	25	9	14	22	18	23	10	11	26	15	12	17
LAOF	Ochsner Foundation Hospital	17	14	14	1	2	3	29	28	22	35	28	30	25	21	18	24	15					8
MAMG	Massachusetts General Hospital	41	84	67	52	21	14	32	27	37	36	40	15	15	21	10	10	16	15	7	7	12	7
MAPB	Brigham and Women's Hospital	48	56	65	59	35	40	47	45	48	75	49	26	34	20	30	27	34	32	30	22	14	24
MIHF	Henry Ford Hospital	24	20	27	30	33	20	22	32	31	26	19	26	16	13	14	8	11	11	12	12	10	7
MISH	Corewell Health Grand Rapids Hospitals	44	62	51	45	41	40	48	42	31	26	22	26	14	13								
MIUM	University of Michigan Medical Center	20	27	23	24	23	38	55	45	23	24	36	36	28	44	23	22	31	36	31	22	29	32
MNMC	Mayo Clinic Hospital Minnesota	42	60	78	69	51	32	7	11	8	11	13	10	11	10	5	15	16	16	13	14	15	12
MNUM	University of Minnesota Medical Center, F...	46	59	66	41	41	32	34	45	38	54	54	57	52	40	29	22	24	20	27	31	47	45
MOBH	Barnes-Jewish Hospital	61	67	61	62	60	70	68	103	74	93	83	69	66	58	67	74	53	57	58	53	58	54
NJBI	Newark Beth Israel Medical Center	24	19	16	14	12	10	14	24	15	16	23	10		2	7	17	6	9	3			
NYCP	NY Presbyterian Hospital/Columbia Univ. M...	78	101	121	106	102	97	59	83	82	71	65	67	60	62	67	59	51	57	46	55	51	43
NYMA	Montefiore Medical Center	16	26	25	17	22	23	26	40	29	5												
NYMS	Mount Sinai Medical Center	22	24	25	24	15														5	11	11	9
NYNS	North Shore University Hospital/Northwell...	29	27	19	13	6																	
NYUC	NYU Langone Health	58	74	76	74	81	53	28	40	24													
NCDU	Duke University Hospital	61	104	94	74	107	108	142	127	97	104	96	106	108	135	108	145	120	93	71	55	60	67
NCMH	University of North Carolina Hospitals	6	12	7	11	8	14	15	24	18	18	14	10	12	7	11	9	15	16	15	10	14	13
OHCC	The Cleveland Clinic Foundation	105	143	139	128	101	95	115	122	113	128	110	92	104	98	100	105	117	154	57	69	62	64
OHOU	Ohio State University Medical Center	55	75	70	54	57	76	68	60	50	42	34	33	18	4				2	1	7	4	11
OKBC	Integris Baptist Medical Center	24	22	24	21	11	13	19	28	36	28	31	22	20	15	17	17	12	14	13	10	9	18
PAPT	University of Pittsburgh Medical Center	36	59	49	58	52	48	74	84	92	59	66	68	86	100	100	108	131	122	118	123	93	87
PATU	Temple University Hospital	111	178	133	110	124	124	131	145	144	131	101	72	56	27	14	5	11	26	39	35	9	15
PAUP	Hospital of the University of Pennsylvania	33	55	66	57	54	59	62	80	112	83	102	72	42	44	45	51	51	45	43	53	51	59
TNVU	Vanderbilt University Medical Center	108	149	97	93	67	54	41	38	41	23	20	27	25	24	15	20	24	16	11	8	18	21
TXHH	Memorial Hermann Hospital, University of ...	10	14	13	23	10	20	20	8	5	7	9	1										
TXHI	CHI St. Luke's Health Baylor College of M...	80	102	108	73	64	71	44	46	38	58	32	15	22	18	14	6	9	9	3	9	5	4
TXMH	Houston Methodist Hospital	80	83	90	81	68	71	61	61	65	73	61	75	100	98	143	108	105	79	51	40	36	22
TXSP	UT Southwestern Medical Center/William P....	59	69	71	68	58	55	46	63	68	78	77	63	62	66	65	50	24	13	21	18	14	21
TXTX	Baylor University Medical Center	39	41	43	31	31	23	36	33	42	41	40	31	36	29	16	29	11	9	8	14	12	20
UTMC	University of Utah Medical Center	31	50	36	31	36	19	10	9	17	22	13	17	15	16	10	13	9	6	6	14	4	6
VAFH	Inova Fairfax Hospital	36	59	49	46	39	29	27	32	26	23	21	19	27	28	24	19	24	26	21	23	36	17
WAUW	University of Washington Medical Center	40	64	61	59	48	53	56	60	51	50	52	53	44	56	41	46	52	46	46	52	52	42
WISE	Froedtert Memorial Lutheran Hospital	17	22	1	6	21	3		3	13	11	9	13	10	11	4		2	4		4	3	5
WIUW	University of Wisconsin Hospital and Clin...	58	66	57	56	42	36	31	44	20	27	28	27	32	37	36	34	37	38	34	25	29	43

US Lung Transplant

At a Glance · October 2026 Update

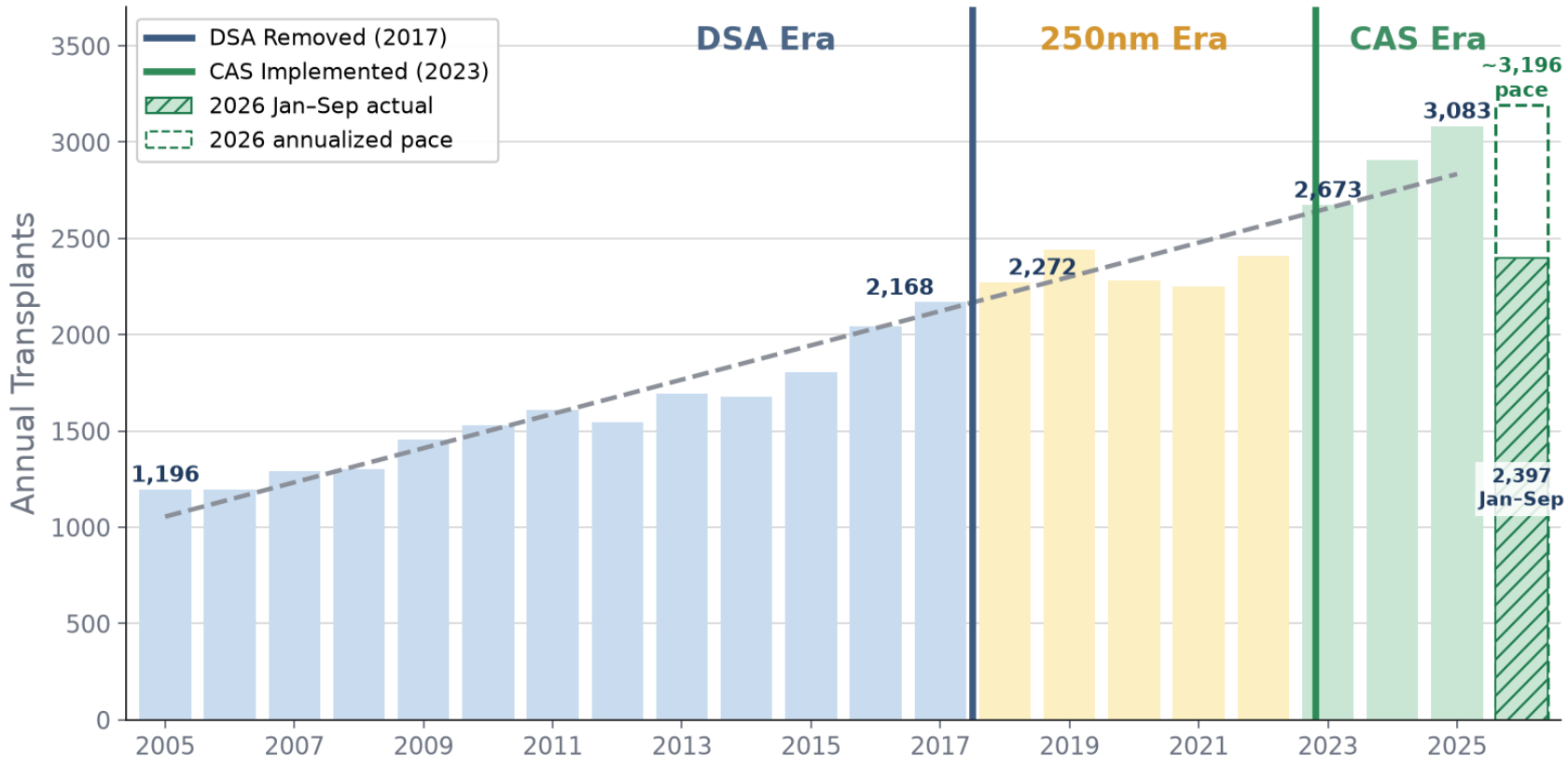
National Insights · ALUA Program Analysis · Policy Impact · Forecasts

3,083	2,397	31	23	90.5%
2025 National Volume	2026 Jan–Sep (US)	ALUA 2025 Volume	ALUA 2026 Jan–Sep	ALUA 1-yr Survival (PSR Jul '26)

OPTN 2005–Sep 30, 2026 (as of Oct 25, 2026) · SRTR PSR July 7, 2026 · Rolling reports Oct 4, 2026 · Offer acceptance Jan–Aug 2026

Part 1 — National Market

US Lung Transplant Volume 2005–2026 (Jan–Sep) with Policy Inflection Points



Era Averages

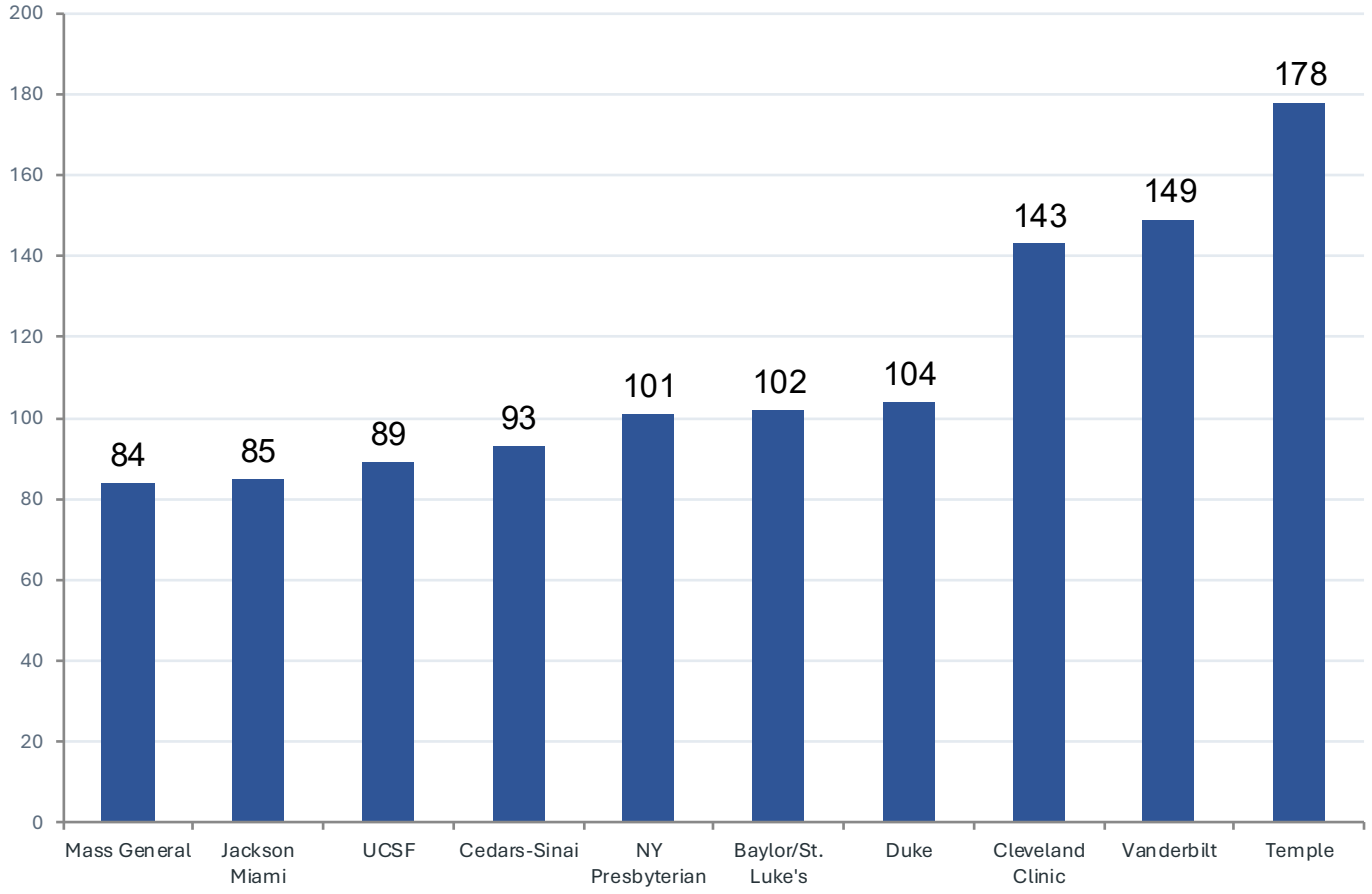
Pre-DSA (2015–17): 2,006/yr

250 NM Era (2018–22): 2,332/yr (+16%)

CAS Era (2023–25): 2,888/yr (+24%)

- **158% growth: 1,196 → 3,083 transplants over 20 years**
- 2023: largest single-year jump (+10.9%) — CAS effect
- COVID 2020 dip (−6.5%) recovered fully by 2022
- Post-CAS slope: +216/yr vs long-run +89/yr
- 2026 Jan–Sep: 2,397 → ~3,196/yr pace (+4% vs 2025) — growth slowing after post-CAS surge

Competitive landscape: concentration is moderate, but leaders matter



Top 10 share of 2025 national volume

36.6%

Largest center

Temple 178

Top three combined

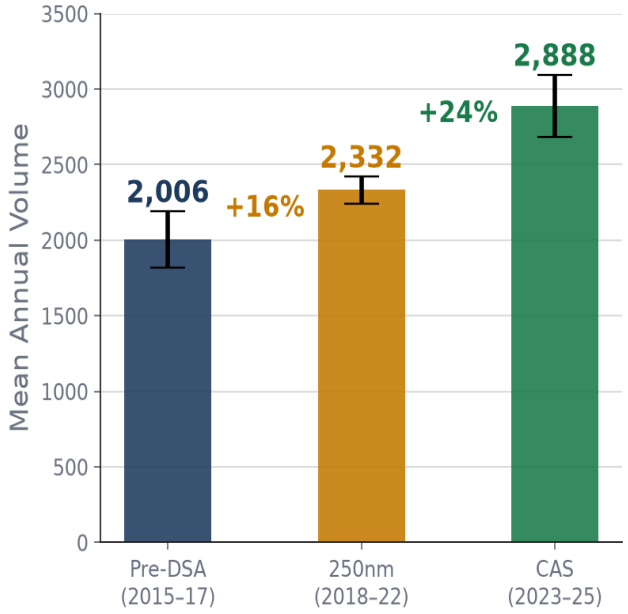
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Market is not monopolistic, but center-specific capability matters.
Leaders combine referral depth, high-acuity candidate management, donor acceptance, and surgical throughput.
ALUA competes in a system where the benchmark is no longer only local geography.

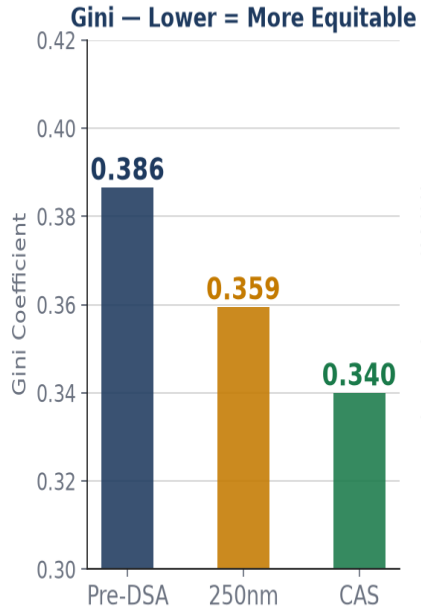
Sources: OPTN/HRSA center data (scraped Oct 26, 2026), SRTR PSR Jan 2026, Rolling SRTR Apr 2026, project analysis.

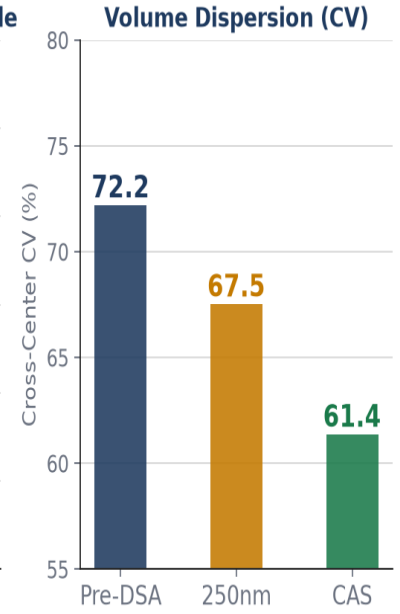
Policy Era Comparison & Equity Impact

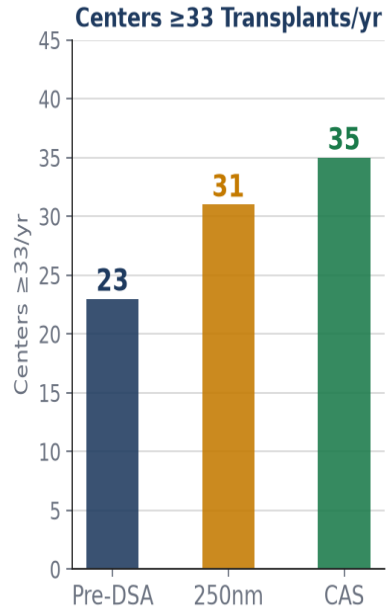
DSA Removal (Nov 2017) · CAS Implementation (Mar 2023)









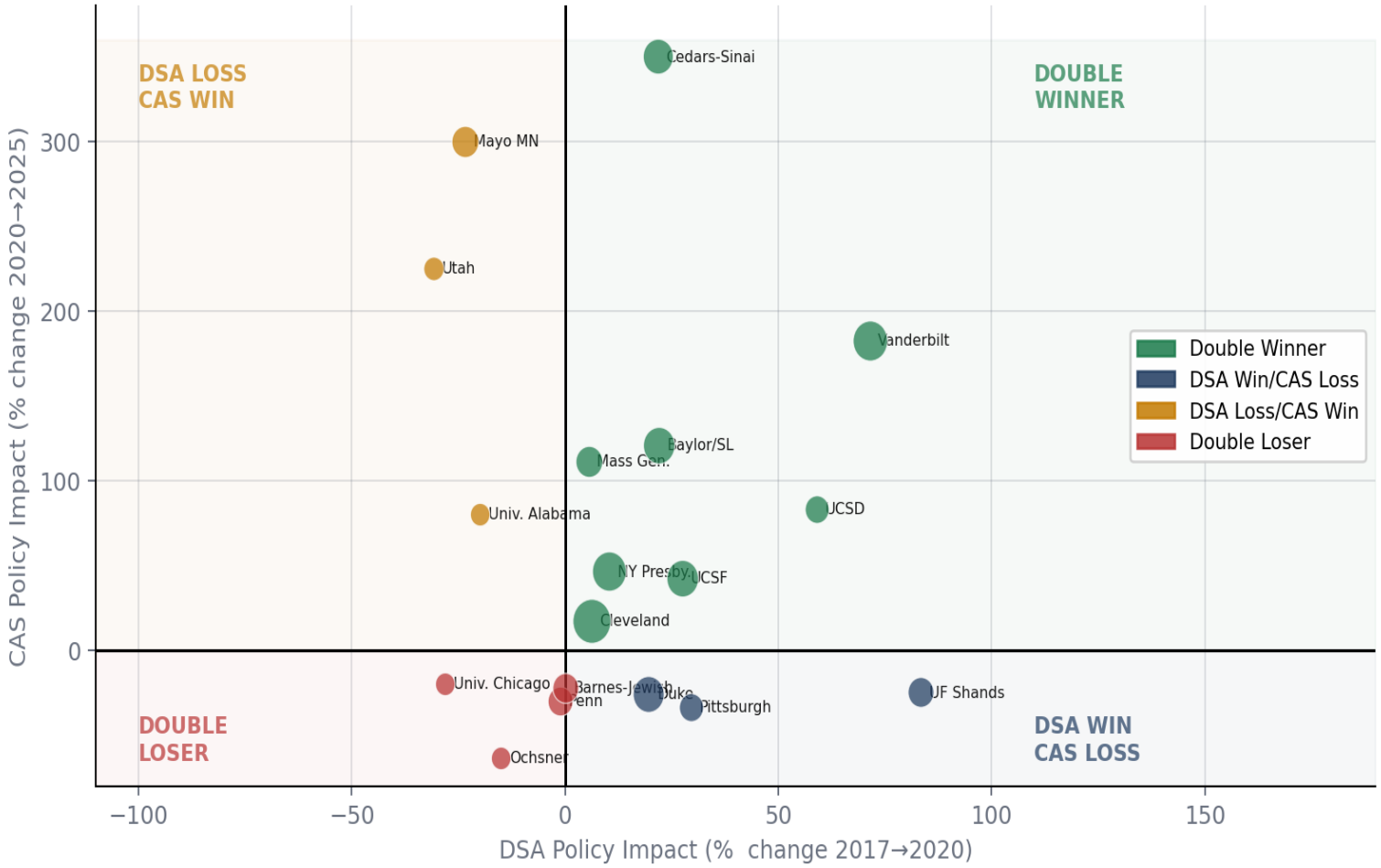


Key Policy Findings

- DSA removal: median waitlist time fell 9 days; transplant rate rose nationally; LOS increased 2 days (transport distance)
- **CAS (Mar 2023): acuity-first composite scoring. Waitlist mortality: 11.2% → 8.4% → 4.1%. Largest YoY volume jump on record.**
- Gini: 0.386→0.359→0.340. CV: 72.2%→67.5%→61.4%. Centers ≥33/yr: 23→31→35 (recomputed, Oct 2026 data). System measurably more equitable under CAS.
- Both reforms market-expanding: only 4 of 51 centers declined under BOTH policies (the Double Losers).

Policy Quadrant — All 51 Centers

Which programs won or lost under each allocation reform?



Double Winner (20)

Benefited from both reforms
Ex: Vanderbilt +384%, Cedars-Sinai +461%

DSA Win / CAS Loss (16)

Geographic expansion; lost on acuity
Ex: Duke −11% net, Pittsburgh −14%

DSA Loss / CAS Win (11)

Recovered under acuity scoring
Ex: Mayo MN +509%, Utah +125%

Double Loser (4)

Both reforms hurt — structural risk
Ex: Ochsner −69%*, Penn −31%
*Ochsner rebounding: 2026 pace ~23

Top Centers & Double Loser Profile

Center	2025 Vol	CAS Avg	Net Δ
Temple University	**178**	140	**+39%**
Vanderbilt	**149**	113	**+384%**
Cleveland Clinic	**143**	137	**+24%**
Duke	**104**	91	**−11%**
Baylor/St. Luke's	**102**	94	**+170%**
NY Presbyterian	**101**	109	**+62%**
Cedars-Sinai	**93**	86	**+461%**
UCSF	**89**	95	**+82%**
Jackson Miami	**85**	75	**+433%**
Mass General	**84**	68	**+123%**

DOUBLE LOSERS

Ochsner (LA) 31→10 avg **−69%**

Below quality threshold, but rebounding: 17 in Jan–Sep 2026 (~23/yr pace) vs 14 in 2025.

Penn/HUP (PA) 86→59 avg **−31%**

Temple (same city, #1 national) dominates on CAS urgency score; Penn loses intra-city share.

Barnes-Jewish (MO) 82→63 avg **−22%**

Midwest OPO geographic premium eliminated; acuity scoring doesn't compensate.

Univ. Chicago (IL) 21→12 avg **−42%**

Loyola in same metro — neither policy rewarded Chicago's geographic position.

PART 2

ALUA Program Analysis

University of Alabama Hospital · Center Code: ALUA · Birmingham, Alabama

31	90.5%	1.13	1.23	23 cases
2025 Volume	1-yr Survival (PSR Jul '26)	PSR Jul 2026 1-yr HR	Offer Accept Ratio (Jan–Aug '26)	2026 Jan–Sep

SRTR PSR July 7, 2026 (data through Apr 30, 2026) · Rolling reports Oct 4, 2026 · OPTN volume through Sep 30, 2026

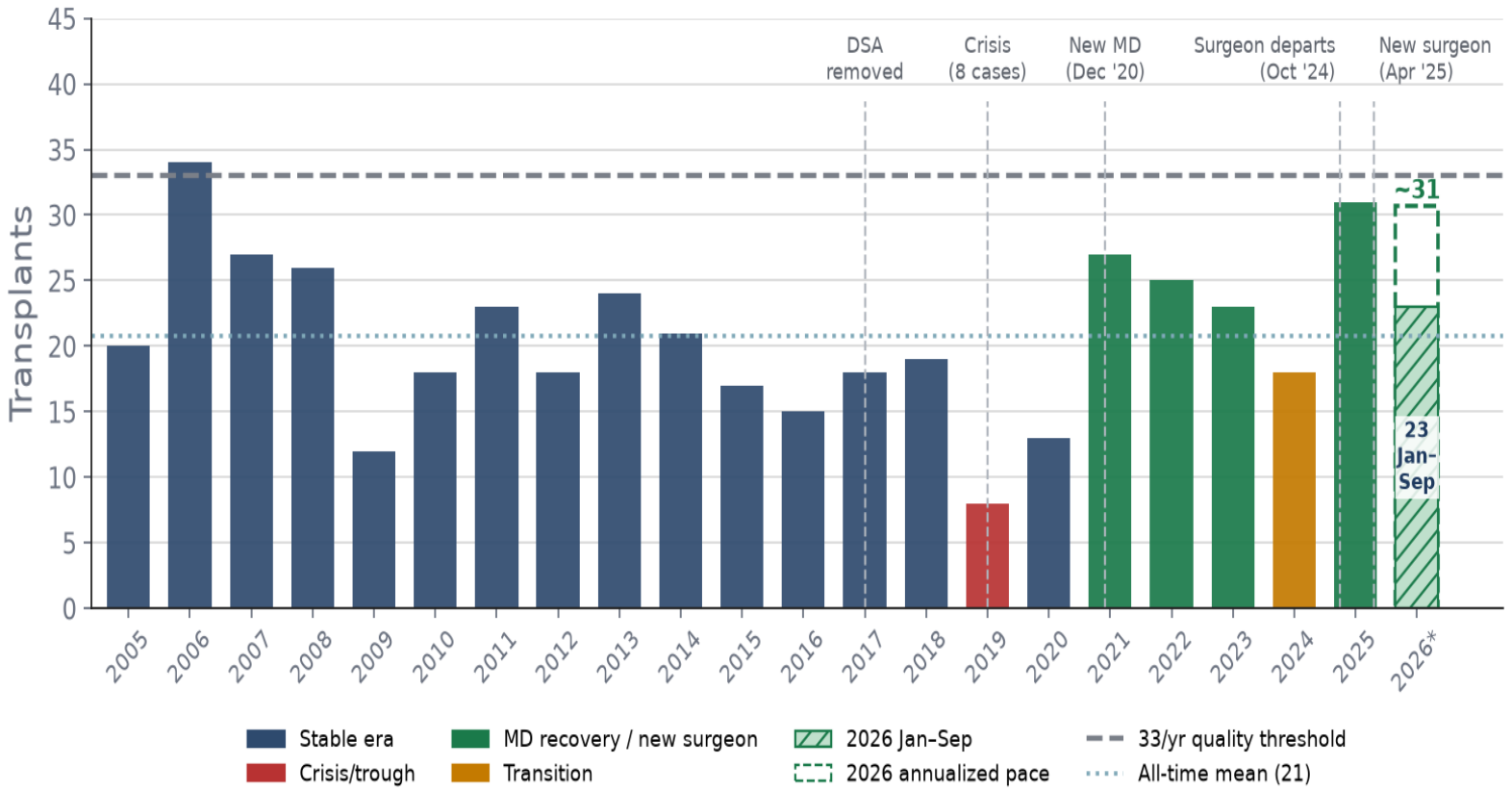
1. Historical Volume & Southeast Peers
2. SRTR Outcomes (PSR Jul 2026)
3. Rolling HR Surveillance
4. Offer Acceptance Analysis
5. Regression Forecast 2026–30
6. Monte Carlo Probability
7. Clinical Profile, UNOS Benchmarks & Recommendations

Key Insights Dashboard

Waitlist	**Waitlist Additions** YTD | Prior YTD **26 | 30**	**Time on Waitlist** Average in days **155.7**	**# | % Active on Waitlist** 9/30/2026 waitlist **11 | 100.0%**	**Waitlist Deaths** Last PSR Cohort | Prior PSR Cohort **4 | 7**
	# | % Active Likely to Transplant 9/30/2026 Waitlist **0 | 0.0%**	**Median CAS at Listing** YTD | Prior YTD **22.66 | 25.74**	**# Non-Transplant Removals** Year to Date **1**	**# | % Inactive Likely to Tx** 9/30/2026 Waitlist **0 | 0.0%**
Transplants & Donors	**# Transplanted** YTD | Prior YTD **26 | 27**	**Avg CAS at Transplant** YTD | Prior YTD **36.0 | 36.3**	**Days to DD Tx** Your Center | Your DSA Rolling 12 Months **101.6 | 101.6**	**% Transplanted within 250 miles** YTD | Prior YTD **25% | 12%**
	# Machine Perfusion Rolling 12 Months **0**	**% DCD Transplants** YTD | Prior YTD **23% | 11%**	**HCV NAT+ Donors** YTD | Prior YTD **1 | 1**	**# NRP** Rolling 12 Months **4**
Organ Offers	**Primary Acceptance Rate** YTD | Prior YTD **14.3% | 3.6%**	**% Local Transplants** Rolling 12 Months **16.7%**	**Avg Ischemic Time | Avg Distance** Hours | Miles : Rolling 12 Mos **6.45 | 295.54**	**Offers Declined & Tx Elsewhere** # | % Rolling 12 Months **186 | 21.0%**
Outcomes	**Graft Loss** 90 Days | 1 Year Conditional Last SRTR Cohort **5 | 2**	**Patient Deaths** 90 Days | 1 Year Conditional Last SRTR Cohort **5 | 2**	**Median Length of Stay** Rolling 12 Months **18.0 Days**	**% PNF** Rolling 12 Months **6.7%**

ALUA — Historical Volume 2005–2026 (Jan–Sep)

Structural event analysis across leadership transitions and policy changes



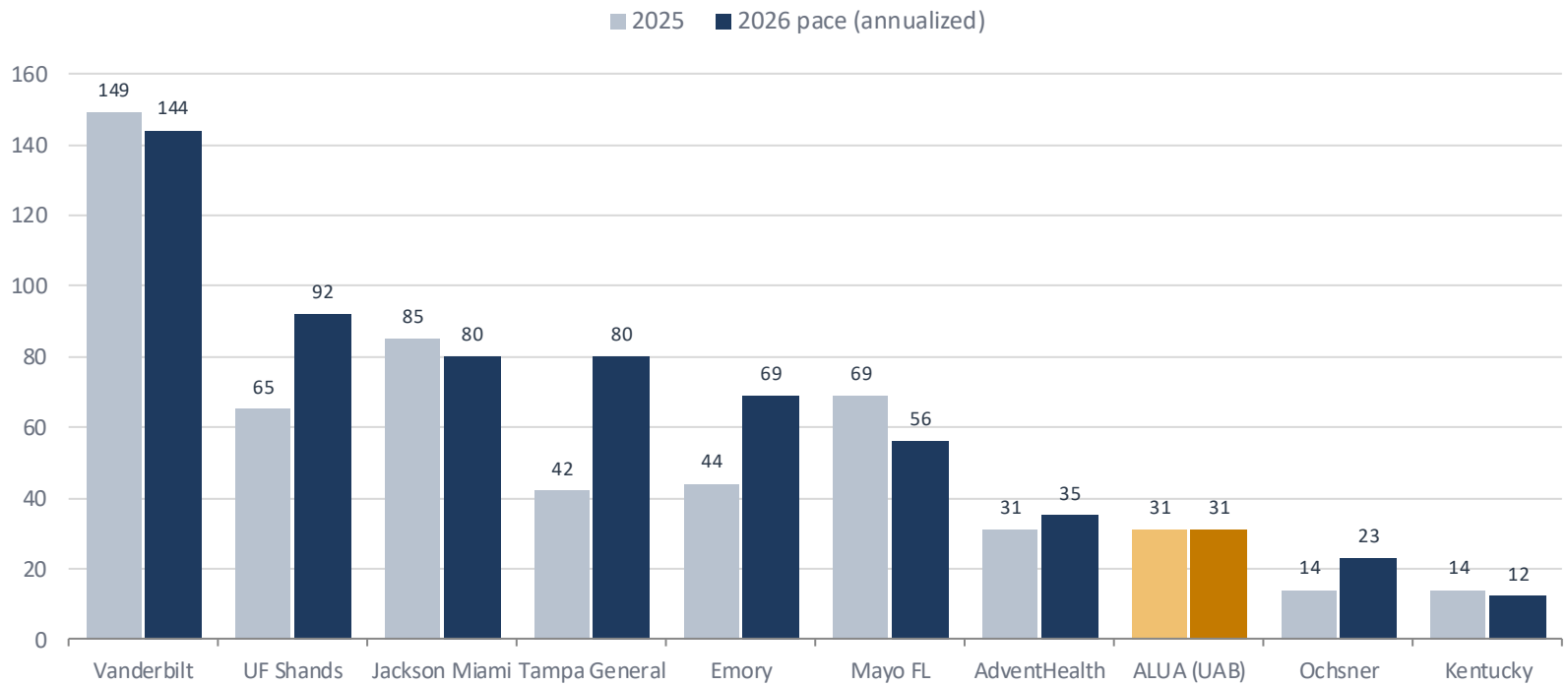
*2026 = Jan 1–Sep 30 (OPTN data as of Oct 25, 2026)

Structural Events

2006	**Peak: 34 cases**	All-time high
2017	**DSA removed**	−20% vs peer +10.5%
2019	**Leadership crisis**	Trough 8 cases (−58%)
Dec 2020	**New MD joins**	+11.2/yr effect (p=0.024)
2021–23	**Recovery era**	27→25→23 cases/yr
Oct 2024	**Surgeon departs**	−22% dip → 18 cases
Apr 2025	**New surgeon**	31 cases — best since 2007
2026 YTD	**23 cases (Q1: 9)**	Pace ~31/yr = 2025 level

ALUA vs Southeast Peers — 2026 Volume

Jan–Sep 2026 annualized (×12/9) vs 2025 · 10 Southeast programs in the 54-center dataset



~31
ALUA 2026 pace

4.9%
ALUA SE share (5.7% '25)

+14%
Southeast 2026 pace

+4%
US 2026 pace

2.3×
Emory vs ALUA (Jan–Sep)

43rd
ALUA US rank (37th in '25)

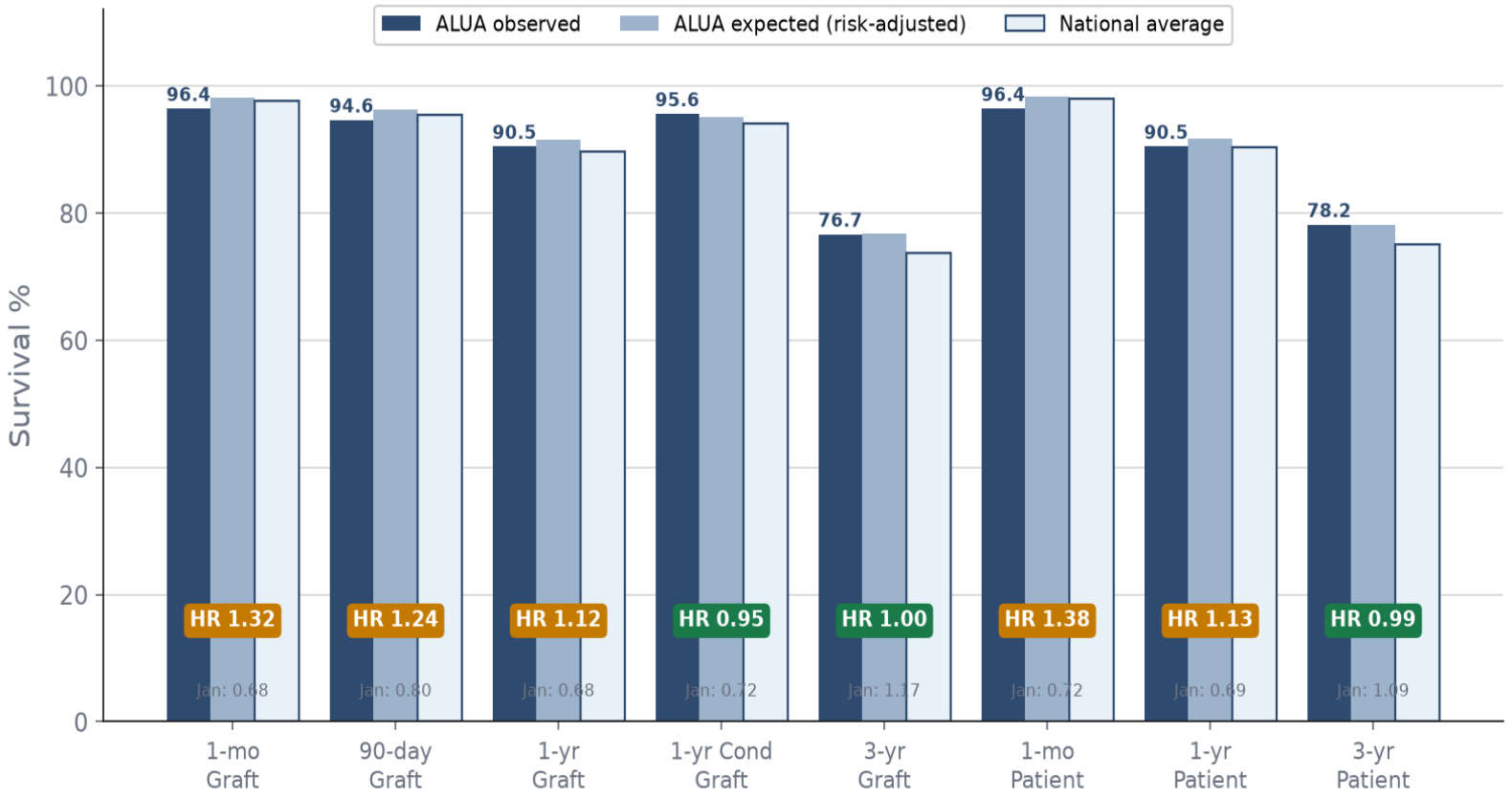
Not in dataset: MUSC, Methodist Memphis, Jewish Louisville, Cleveland Clinic FL, Wellstar MCG.

Regional Insights

- **Region growing fast: the 10 programs are on pace for ~621 in 2026 (+14% vs 544 in 2025), vs +4% nationally.**
- ALUA flat at ~31 (2025: 31) → regional share 5.7% → 4.9%; national rank 37th → 43rd of 54.
- Emory (Atlanta, ~150 mi): 52 by Sep is already a program record; pace ~69, more than 2× ALUA after matching it in 2020–24 (~21/yr each).
- Vanderbilt (Nashville, ~190 mi) is levelling off at ~144 but is still ~4.7× ALUA. Growth is now led by Tampa, Emory and Shands.

ALUA SRTR Outcomes — PSR July 2026

Cohort: Jan 2023–Jun 2025 · n=56 · Outcomes now at expected (Jan 2026 PSR: HR 0.69) · credible intervals remain wide



90.5%
1-yr Graft Survival (exp. 91.5%)

1.12
1-yr Graft HR [0.45–2.09]

90.5%
1-yr Patient Survival (exp. 91.7%)

1.13
1-yr Patient HR [0.46–2.11]

76.7%
3-yr Graft Survival (exp. 76.7%)

1.00
3-yr Graft HR [0.56–1.56]

5 obs / 4.26 exp
1-yr Graft Failures vs Expected

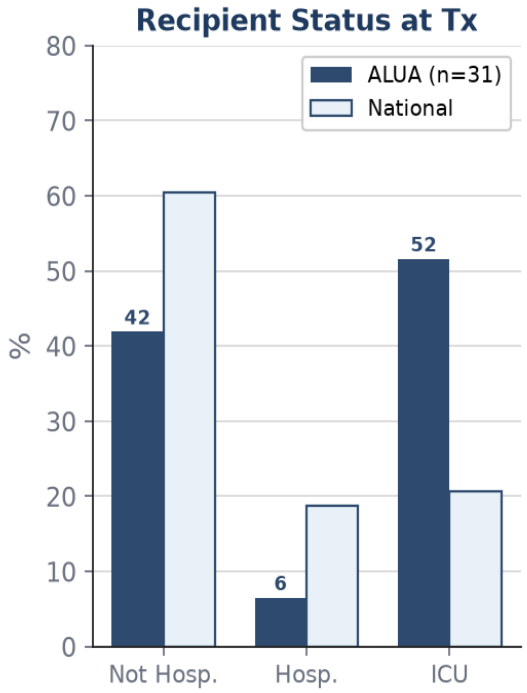
★ **90-day graft HR 1.24 (3 vs 2.03 exp.) · 1-yr conditional HR 0.95 — all HRs well below the MPSC threshold (2.25)**

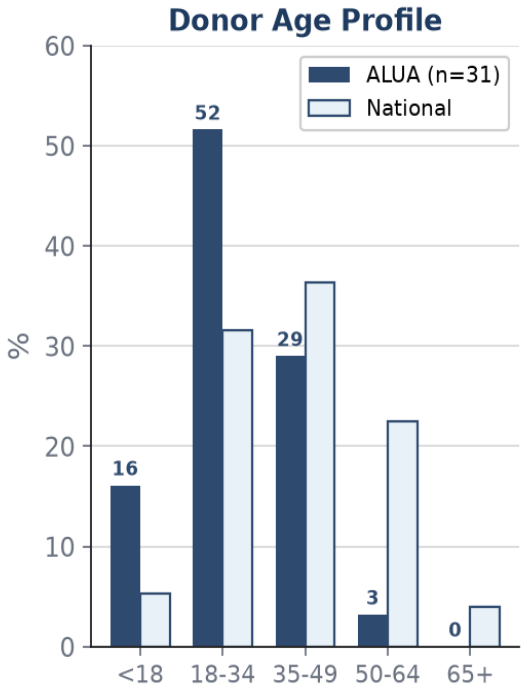
△ Jan → Jul 2026: 1-yr HR 0.69 → 1.13, survival 95.7% → 90.5%; the added failures are 2025 early-graft losses

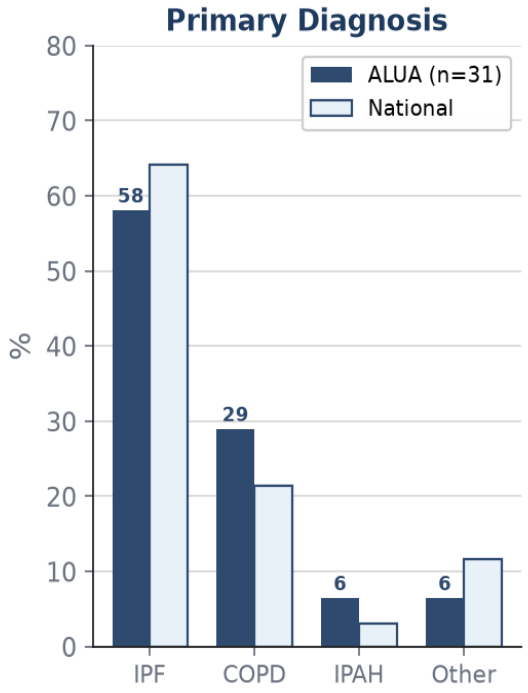
- National 1-yr: 89.8% graft / 90.3% patient — ALUA now at the national rate and within expected

ALUA Clinical Profile

Recipient & donor characteristics vs national benchmarks · Jan–Dec 2025 · n=31 · PSR July 2026





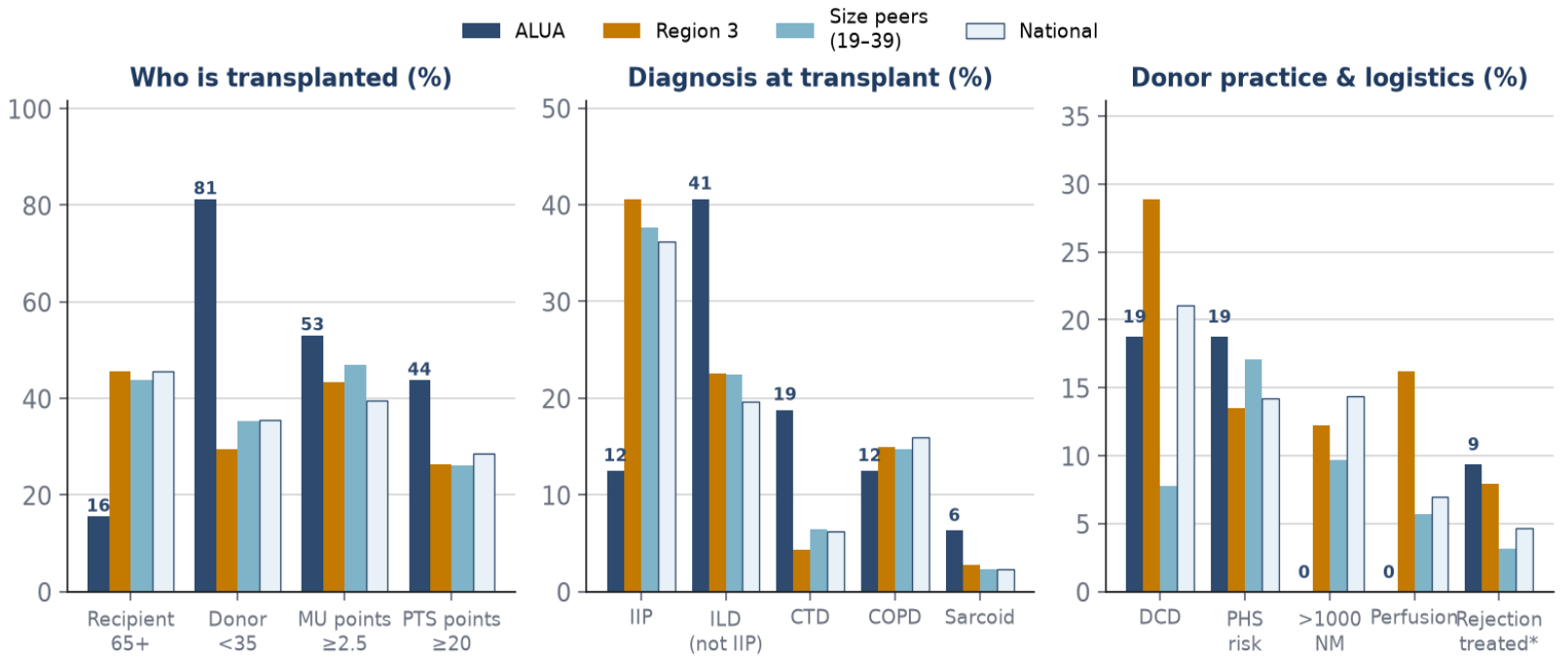


Clinical Insights

- **ICU at transplant: 51.6% vs national 20.7% (region 42.8%) — 2.5× national acuity intake, up from 46.2%. CAS incentivizes urgent patients; this raises early-failure risk.**
- Donor age: 51.6% aged 18–34 vs 31.6% nationally; 16.1% under 18; only 3.2% aged 50+ (national 26.5%). Young donors remain the main protective factor.
- Diagnosis: COPD 29.0% vs 21.4% national; IPF 58.1% vs 64.1%. Recipient BMI ≥36: 9.7% vs 1.2% national.
- Post-tx LOS: median 24 days vs 21 national (region 26). Median time to transplant 3.4 mo vs 1.3 national; 83.9% of donors from outside the DSA.

ALUA vs UNOS Benchmarks — Region 3, Size Peers, National

UNOS Benchmark Report Oct 2026 · transplants Jul 2025–Jun 2026 (n=32) · waitlist snapshot Sep 30, 2026 (n=11)



*acute rejection treated before discharge · UNOS Benchmark Report Oct 2026

32
Tx Jul '25–Jun '26 (US median 40)

6.4 h
Median ischemic time (US 9.8)

16%
Recipients 65+ (US 46%)

81%
Donors under 35 (US 36%)

9.4%
Treated rejection pre-discharge (US 4.7%)

0%
Machine perfusion (Region 3: 16%)

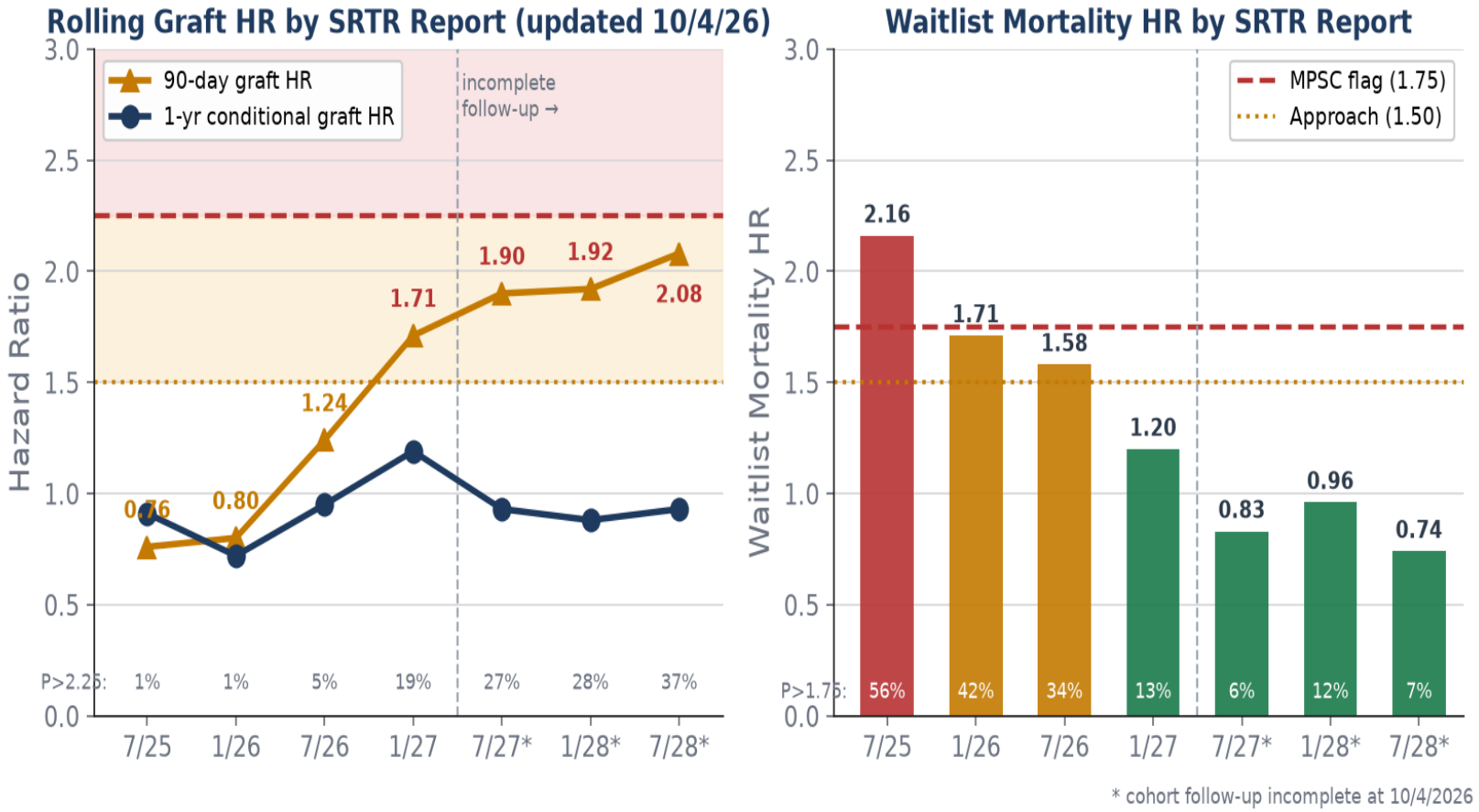
Size peers = Q2:19–39 transplants/yr (19 programs); Region 3 = 8 programs. MU = medical urgency; PTS = post-tx survival points.

Benchmark Insights

- **Young on both sides: recipients 65+ are 15.6% of ALUA transplants vs 45.7% nationally; donors under 35 are 81% vs 36% (no donors 50+). The waitlist is 9% aged 65+ vs 49% nationally — the largest untapped listing segment.**
- Distinct case mix: non-IIP ILD 40.6% + connective-tissue disease 18.8% + sarcoid 6.3% = 66% vs 28% nationally; IIP only 12.5% vs 36.2%. Urgency is high (MU points ≥2.5: 53% vs 40%) yet predicted post-tx survival is better (PTS ≥20: 44% vs 28%).
- Conservative logistics protect early outcomes but cap the offer pool: median ischemic time 6.4 h vs 9.8 national (14.0 Region 3); no donors beyond 1,000 NM (14% nationally); 100% ABO-identical; 0% machine perfusion vs 16% in Region 3.
- Watch: acute rejection treated before discharge 9.4% vs 4.7% nationally (3.2% size peers) — consistent with the 90-day graft signal. Recipient BMI median 29.3 vs 26.3; median LOS 21 d vs 20 (Region 3: 27).

Rolling SRTR HR Trend — Surveillance Update (Oct 4, 2026)

SRTR 7/26 confirmed at 90-day graft HR 1.24 (April projection 1.25); 1/27 cohort enters approach zone at 1.71 — early graft failure, not 1-yr survival



* cohort follow-up incomplete at 10/4/2026

⚠ APPROACHING FLAG — 90-DAY GRAFT

7/26 actual 1.24 → 1/27 1.71 (5 failures vs 2.1 exp.)
7/27*–7/28* tracking 1.90–2.08 | P(HR>2.25) → 37%
Approach zone 1.50–2.25 | MPSC flag >2.25

✓ WAITLIST MORTALITY RESOLVING

WLM HR 2.16 (7/25) → 1.58 (7/26) → 1.20 (1/27)
P(HR>1.75) 56% → 13%; 7/27*–7/28* at 0.74–0.96
PSR 7/26 (2024–25): pre-tx mortality 1.58 [0.68–2.84], tx-rate ratio 0.59 [0.44–0.76] — still carry 2024 trough

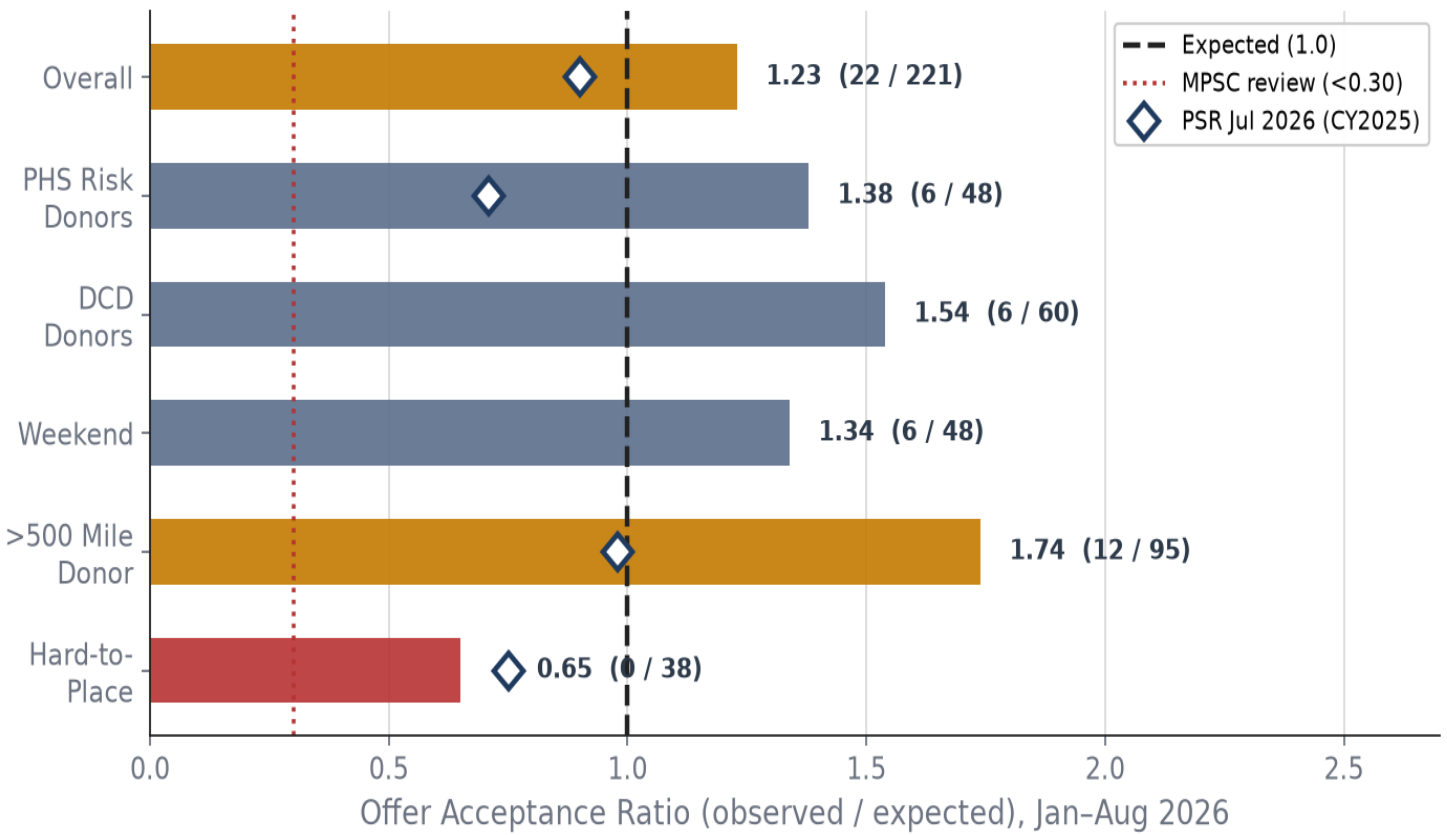
WHAT THE DATA SAY

1-yr conditional graft HR stays 0.88–1.19 — the signal is perioperative (≤90-day) failure
90-day failures: 2/16, 2/15, 1/17, 1/7 (H1'25→H2'26)
ICU intake 52% vs 20.7%; new surgeon from Apr '25

★ **SRTR 1/27 (Jan 2027) will be the first official report in the approach zone — case-review the 2025–26 early graft failures now**

ALUA Offer Acceptance — Lever Pulled (Jan–Aug 2026)

Jan–Aug 2026: 221 offers · 22 accepted · ratio 1.23 — up from 0.90 (CY2025, PSR Jul 2026) and 0.62 (PSR Jan 2026)



Acceptance Strategy Analysis

Overall ratio: 0.62 → 0.90 → 1.23

PSR Jan '26 → PSR Jul '26 (CY2025) → Jan–Aug '26

Accepted 22 / 221 offers (Jan–Aug '26)

CY2025: 31 / 526, 34.5 expected, 0.90 [0.62–1.24]

Hard-to-place: 0.65 (CY2025: 0.75)

0 of 38 accepted — still deferred

>500 miles: 1.74 (CY2025: 0.98)

12 of 95 — distance range has been expanded

PHS risk 1.38 (was 0.71) · DCD 1.54

Both above expected — broader donor acceptance

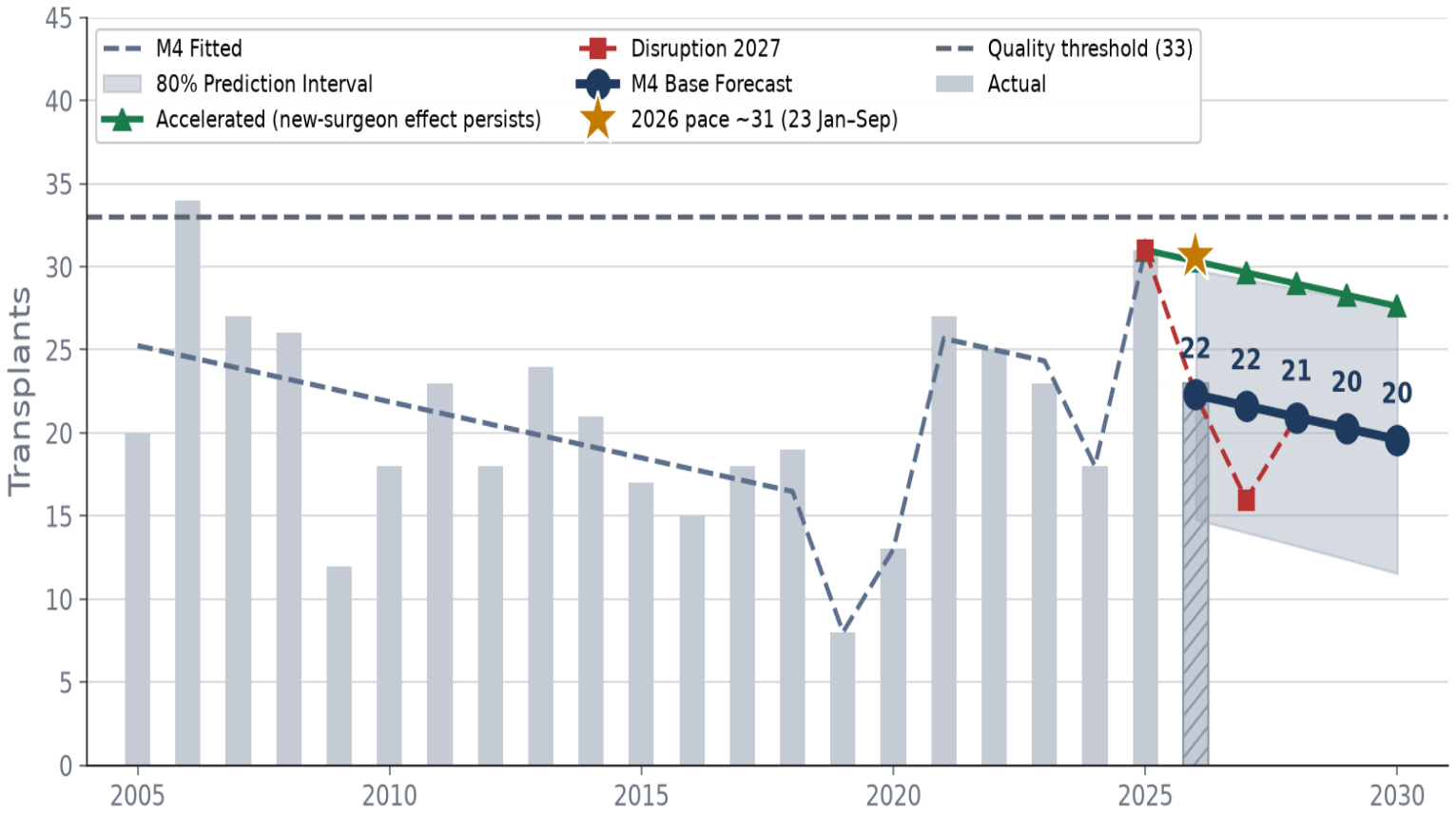
MPSC trigger: <0.30

ALUA at 1.23 — no regulatory risk

Next constraint is listings, not acceptance: active waitlist fell 13 → 8 (Jul '24 → Jul '26); 16–17 transplants per half-year ≈ removals. Grow referrals; watch 90-day outcomes as distance/DCD acceptance widens.

ALUA Volume Regression & Forecast 2026–2030

M4 parsimonious structural model · AIC=70.6 · Adj R^2=0.448 · Lowest AIC of 4 models



Model Comparison

M1: Linear	−0.041	80.3
M2: Quadratic	0.199	75.7
M3: Full struct.	0.327	75.7
M4: Best ★	**0.448**	**70.6**

M4 Forecasts: Base vs Accelerated

2026: base 22 (PI 15–30) · accel. 30

2027: base 22 (PI 14–29) · accel. 30

2028: base 21 (PI 13–29) · accel. 29

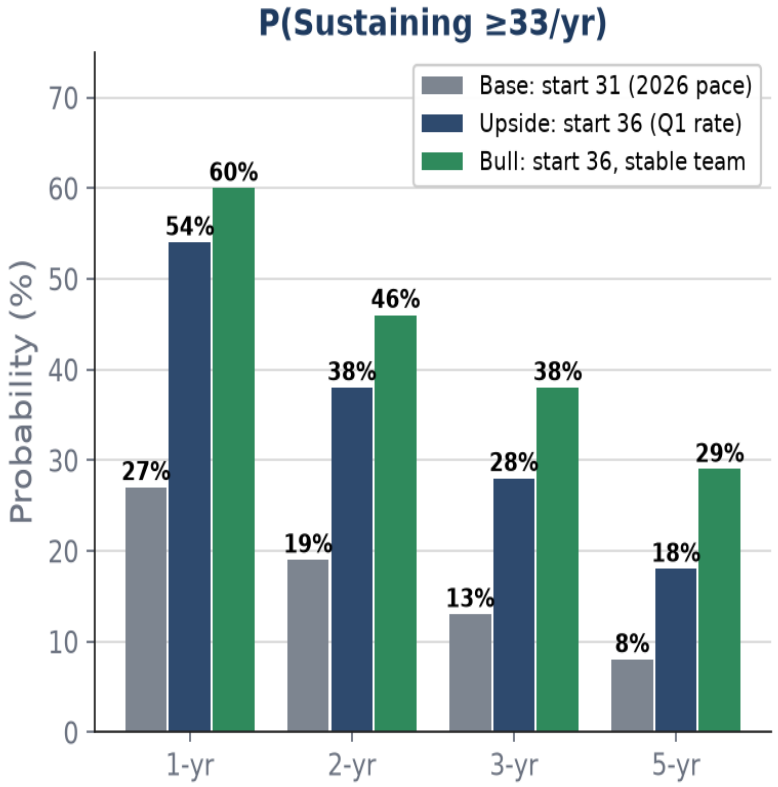
2029: base 20 (PI 12–28) · accel. 28

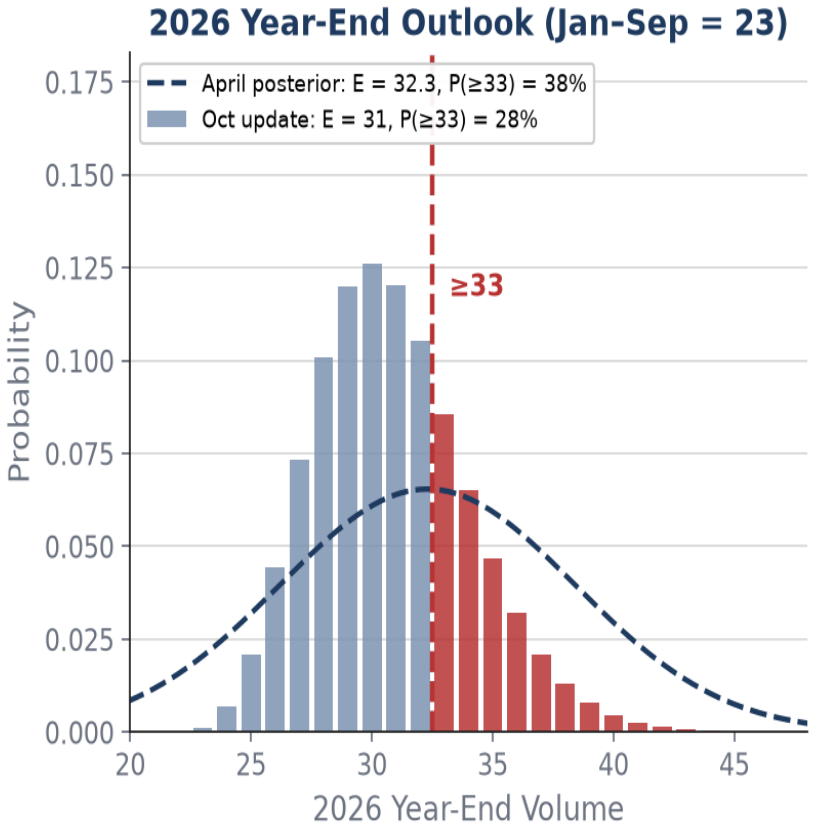
2030: base 20 (PI 12–28) · accel. 28

★ 2026 pace ~31 (23 by Sep): tracking accelerated path

Monte Carlo Probability & Bayesian Update

100k simulated paths · Regime-switching model · Year-end update with Jan–Sep 2026 actuals





P(Sustaining ≥33 cases/yr)

Scenario	1yr	3yr
Base (start 31)	27%	13%	8%
Upside (start 36)	54%	28%	18%
Bull (36, stable)	**60%**	**38%**	**29%**

2026 Year-End Update

Given Jan–Sep 2026 = 23 cases (Q1 = 9):

April: E[vol] = 32.3 | P(≥33) = 38%

Oct: E[vol] = 31 (80%: 27–35) | P(≥33) ≈ 28%

Needs ≥10 in Q4 (best quarter so far: 9)
Retaining surgical team = highest-leverage action

Key Recommendations & Surveillance Priorities

Actionable priorities derived from quantitative analysis of SRTR PSR, rolling data, and volume models

1 **90-Day Graft Failure Review Before SRTR 1/27 (Jan 2027)**

7/26 confirmed at 1.24; the 1/27 cohort is at 1.71 (5 early failures vs 2.1 expected), later windows 1.9–2.1. Case-review every 2025–26 ≤90-day failure now.

2 **Retain Surgical Team Through April 2027**

Disruption probability is the dominant model parameter. Two years at ~31/yr support a durable new-surgeon effect; a disruption resets to ~22.

3 **Shift From Acceptance to Listings**

Acceptance ratio 1.23 (>500 mi 1.74, DCD 1.54) — the April lever is pulled. Growth now depends on listings: 65+ candidates are 49% of the national list but 9% of ALUA's.

4 **Monitor Acuity-Adjusted Metrics**

HRs are risk-adjusted, but ICU intake is now 2.5× national (51.6%) and distance/DCD acceptance just widened. Ask SRTR whether the model under-adjusts at this acuity.

5 **Q4 2026 = Year-End Checkpoint**

Needs ≥10 Q4 cases for 33 in 2026 (P ≈ 28%); realistic ~31. Set 2027 target ≥33; track Emory and Vanderbilt share.

Summary

National Market

3,083 transplants (2025), +158% since 2005; 2026 Jan–Sep 2,397 (pace ~3,196, +4%). Both reforms improved equity and access; CAS produced the largest single-year jump.

Policy Impact

Only 4 of 51 centers lost under both reforms. CAS rewards acuity-optimized programs. Southeast volume is growing fast (+14% 2026 pace), led by Vanderbilt, Emory, Tampa and Shands.

ALUA Current State

PSR Jul 2026: 1-yr survival 90.5% (expected 91.7%), HR 1.13 — normalized from Jan's 0.69. 90-day graft HR 1.24; rolling 1/27 cohort at 1.71 (approach zone) on early failures. WLM resolving (1.20).

ALUA Trajectory

2026: 23 by Sep, pace ~31 — second year at the 2025 level. Offer acceptance 0.62 → 0.90 → 1.23; constraint is listings. P(2026 ≥33) ≈ 28%. SE share 5.7% → 4.9%.

Data: OPTN 2005–Sep 30, 2026 · SRTR PSR Jul 7, 2026 · Rolling reports Oct 4, 2026 · Offer acceptance Jan–Aug 2026 · Analysis: OLS Regression, Monte Carlo, Bayesian Update